

# Inspection of Central Bedfordshire local authority children's services

**Inspection dates:** 10 to 14 March 2025

**Lead inspector:** Rebecca Quested, His Majesty's Inspector

| Judgement                                                                | Grade |
|--------------------------------------------------------------------------|-------|
| The impact of leaders on social work practice with children and families | Good  |
| The experiences and progress of children who need help and protection    | Good  |
| The experiences and progress of children in care                         | Good  |
| The experiences and progress of care leavers                             | Good  |
| Overall effectiveness                                                    | Good  |

Since the last inspection in 2022, when children's services were judged to be good, Central Bedfordshire has continued to provide good services for children and families. The areas for development identified at previous inspections have been successfully addressed and other services positively developed. The local authority has prioritised investment in support for the most vulnerable children and the new lead member and director of children's services (DCS) have continued to champion the experiences of children across the council.

Improvements in services are making a tangible difference to children and families. Particular strengths are relational social work practice with families and the quality of direct work with children. Effective services to young carers, those at risk of exploitation and those children aged 16 or 17 who present as homeless provide specific support to these vulnerable groups of children. Most children in care are making progress and are living in safe and stable homes, and care leavers receive sensitive support from their personal advisers (PAs) as they transition to adulthood.

Senior leaders are well informed by a detailed and accurate self-evaluation which identifies areas where further strategic attention is needed, and there are firm plans in place to address these.

## **What needs to improve?<sup>1</sup>**

- The effectiveness of the emergency duty team (EDT) in supporting children at night and at weekends. (outcome three, national framework)
- The timeliness of permanence decisions for some children in care. (outcome four, national framework)
- The participation of children in care and care leavers in service development. (enabler two, national framework)

## **The experiences and progress of children who need help and protection: good**

1. Services for children who need help and protection in Central Bedfordshire are strong. Since the last inspection in 2022, there have been improvements in the timeliness and oversight of the pre-proceedings stage of the Public Law Outline (PLO) and in developing a multi-agency safeguarding hub known as the integrated front door (IFD), with more partner agencies now co-located. Skilled practitioners in the IFD undertake detailed screening and safety planning to deliver effective analysis that is helping to reduce harm for children. The threshold of need and risk of harm are appropriately identified and responded to.
2. Children requiring targeted support are promptly assigned to locality-based practitioners and family help partners who work closely with social workers to deliver child-centred support that helps children remain safely with their families. Skilled early help practitioners use a variety of specific tools to support the development of relationships with parents and children and help them to express their wishes and concerns.
3. The quality of information about children and their experiences provided by partner agencies to the IFD is variable and referrals from the police are often delayed. For a few contacts, there are significant gaps in basic information, including names and dates of birth, or clear details about the concerns about children. Children's voices and experiences are too often missing. This is creating additional pressure and work for practitioners in the IFD and avoidable delay in decisions for some children.

---

<sup>1</sup> The areas for improvement have been cross-referenced with the outcomes, enablers or principles in the [Children's Social Care: National Framework](#). This statutory guidance sets out the purpose, principles for practice and expected outcomes of children's social care.

4. Practitioners in the IFD actively engage parents to understand their circumstances, ensuring interventions are tailored to children's needs. Parental consent to share information is obtained or dispensed with appropriately. A daily multi-agency meeting is held in the IFD to review referrals swiftly for children at risk of harm from domestic abuse. Management oversight and direction appropriately identify risks and strengths and provide clear direction to practitioners. Social workers explore concerns sensitively and base decisions on the child's history and experiences, avoiding unnecessary statutory interventions and providing timely support.
5. Child protection strategy meetings are well attended, prompt and collaborative, ensuring that partner agencies share relevant information to inform decision-making. These meetings prioritise the understanding of risks and the identification of family support needs. Child protection enquiries are timely and thorough.
6. The EDT responds appropriately to most requests for support out of hours and social workers attend strategy meetings with police colleagues when required. There is an effective handover between daytime services and the EDT, including information about the availability of resources such as foster care and supported accommodation. Although appropriate action is taken to ensure children's protection out of hours, children are not always visited by social workers at night or at weekends when such support might be helpful, including sometimes when children are subject to police protection.
7. Most assessments of children's needs are sensitively written directly to children and seek to understand the impact of parental mental illness, domestic abuse, substance misuse and neglect on children's well-being and development.
8. Family network meetings are used effectively across the Family Help service to develop support plans to enable children to live safely with their families. Safety plans are co-created with families, which fosters cooperation and realistic, actionable steps to mitigate risk to children. A comprehensive range of local resources supports families who are experiencing domestic abuse, substance misuse and poor mental health.
9. Social workers develop effective child-centred child in need and child protection plans. These are regularly reviewed by the relevant professionals and family members to monitor children's progress and evaluate the level of need and risk. Children are supported to attend child protection conferences and are offered support from an independent advocate if they wish. When children's circumstances or risks change, plans are appropriately escalated or stepped down to ensure that children and their families receive a proportionate level of service and support.
10. Social workers and family practitioners visit children frequently to build trusting relationships. These visits and observations of children are purposeful and

provide good insight into children's lives. Children are supported to talk about their lives and experiences through age-appropriate and effective direct work, using a range of tools. The records of visits are written to the child, which will help them understand how they are being helped should they read their records now or in the future.

11. Significant improvements have been made in the use of the pre-proceedings stage of the PLO since the last inspection. These improvements have ensured that more children are supported to remain living with their families or that proceedings are issued promptly, preventing drift and delay for most children. Letters before proceedings are written with a clear focus on the concerns about children's experiences. Assessments are usually frontloaded before proceedings are issued, preventing unnecessary delays.
12. Disabled children receive support from practitioners who have an in-depth understanding of their specific needs and histories. The use of communication tools helps practitioners engage with children effectively and capture their views. Multi-agency planning and review meetings ensure coordinated support for disabled children, and decision-making regarding the level of intervention is robust. Visual aids and creative approaches assist families in understanding concerns and progressing towards solutions. Consistent care and opportunities for independence are provided, ensuring disabled children benefit from tailored support which improves their experiences.
13. Children at risk of exploitation benefit from timely identification and appropriate support. Staff can readily access specialist advice and support, which they use to better understand and mitigate risks. Partnership working in this area is effective, with sound information-sharing and collaboration. When children have been missing, return home interviews are offered by specialist workers who provide children with the opportunity to discuss their experiences and support their understanding of risk. Children are encouraged to take up the offer of a return home interview and most do so.
14. The local authority designated officer (LADO) service is staffed by experienced professionals who effectively identify and mitigate risks posed by professionals working with children. They work closely with employers, ensuring that any necessary action is taken to protect children.
15. Services to children who are young carers are well developed in Central Bedfordshire. As a result, young carers receive high-quality, holistic assessments of their needs that inform individualised support plans. These children are visited regularly, with plans reviewed at team around the child (TAC) meetings. A range of community-based resources are available to support young carers and services are responsive to their needs. The frequency of visits and the involvement of relevant individuals ensure that the needs of young carers are consistently met.

16. The response to children aged 16 or 17 who are at risk of becoming homeless is well established. A homeless and mediation service practitioner works with children who present as homeless. Children are provided with advice on all the options available to them, including becoming looked after. Advocacy is offered to all children and there is a high take-up of this service. This supports children to consider their options carefully. Family meetings are used effectively in reaching solutions. As a result, most children are successfully supported to remain living with their families.
17. Senior leaders recognised that children in private foster care were not being assessed and visited within appropriate timescales. In the last few months, responsibility for private foster care has moved to the fostering service and there has been a thorough review of all privately fostered children's circumstances. New referrals of children who are subject to private fostering arrangements are now handled in a timely way by the fostering team and assessments processed swiftly. Awareness of private fostering is promoted in the community through meetings and advertising, and practitioners in the IFD and across the teams receive regular refresher training to ensure they have up-to-date knowledge.
18. Social workers say their managers are accessible and approachable. Most receive regular, helpful supervision that supports children's plans to progress. Managers provide oversight on children's case files and are actively involved in supporting social workers, such as by chairing strategy and complex case discussions.
19. There is effective oversight of children who are missing education. This includes thorough safeguarding checks and systems to trace and locate those children who are missing education.
20. Central Bedfordshire has a suitable policy for those children who are electively home educated. Information in the policy is clear for parents and carers to understand, and sets out the roles and responsibilities of parents, schools and the local authority. All reasonable measures are taken to consider the vulnerability of children who are electively home educated, and they are visited in line with their identified vulnerability.

## **The experiences and progress of children in care: good**

21. Most children come into care when they need to, with decisions overseen by senior managers scrutinising the right information to ensure that action is taken in children's best interests.
22. Family group meetings are used early in a child's journey to ensure the involvement of extended family in caring for children who cannot safely stay at home. Children live with their brothers and sisters whenever possible. When

this is not possible, there is careful consideration of the family time they spend together to support these important lifelong relationships.

23. Most children in care in Central Bedfordshire thrive in stable homes. Social workers support them through developing strong, child-focused relationships.
24. Most children's case records, including their plans, are well written and permanency planning is considered promptly. Records of these discussions are often brief and the rationale for decisions is not always clear. This could create difficulties for children in understanding their care journeys. Children's plans are regularly reviewed by independent reviewing officers (IROs), who are effective in ensuring that children's views are central to decision-making, with IRO oversight clearly reflected in children's records. IROs write sensitive review minutes to children to ensure they have a clear and accessible record to support their understanding about the decisions that are made about their lives.
25. Purposeful and detailed assessments for disabled children in care support planning which demonstrates a thorough understanding of their needs. Children's voices are captured well, using a range of communication tools. Effective early collaboration with adult services makes sure that children who will require lifelong support are identified at the right time and benefit from timely joined-up care planning for their transition to adulthood.
26. Strong child-centred assessment, planning and support take place for children who are returning to live with their parents. This promotes children being safely reunited with their families at a time which is right for them and with the appropriate support in place to meet their needs.
27. Children in care are visited regularly by social workers who build trusting relationships and act on their wishes. This includes visits to those children who are living outside Central Bedfordshire, to meet their individual specific needs. Social workers speak with pride and knowledge about the children they support and the progress they make.
28. Unaccompanied asylum-seeking children are described as 'children who are seeking sanctuary'. They are well supported by social workers who take time to understand their experiences. They are supported to keep in contact with family members whenever possible. Social workers ensure that children have access to legal advice and that their specific and diverse needs are met. As a result, most children seeking sanctuary are settled and making progress.
29. Children in care can access good-quality advocacy and are well supported to participate in their own care planning, but arrangements for their involvement in wider service planning are weak. The current Children in Care Council (CiCC) has a committed but small membership and, while there is consultation with this group, the impact of this is unclear.

30. The needs of children in care who are at risk of exploitation or who go missing are well understood by social workers, who receive expert advice from a dedicated exploitation lead. Strong multi-agency work supports children, with all professionals coming together in well-coordinated meetings to identify ways of helping children and to disrupt sources of harm.
31. Children who are subject to deprivation of liberty through the inherent jurisdiction of the high court are well tracked by senior leaders, in collaboration with the IRO service and social workers. Restrictions are carefully reviewed and when appropriate reduced, reflecting children's changing needs and risks.
32. Children's physical and emotional health needs are well met, with regular reviews ensuring a clear understanding of their needs. Children's carers ensure that their immunisations are up to date and that their health needs are met on a day-to-day basis. Children have good access to child and adolescent mental health services (CAMHS) when this is required.
33. Children in care are making good progress at school, with regular personal education plans (PEPs) maintaining a focus on their needs. The virtual school's expectations about the quality of PEPs and the tangible difference that they should make is continually reinforced, ensuring that they are of a high standard. Children in care benefit from opportunities to try new hobbies and develop new interests and to learn important life skills.
34. Most children in care are well supported to have a sense of security and permanence where they are living. Some experience delay in being formally matched to their long-term foster carers or securing legal permanence in special guardianship arrangements. Leaders have identified that their current dataset and tracking tools need enhancing to provide them with a stronger line of sight. This is planned to promote more timely action in agreeing permanence decisions so that children can confidently integrate into carers' families knowing they will be living with them long term.
35. Overall, the effective recruitment, assessment, support and supervision of foster carers are helping them to care safely for children. The development of a foster carer support hub has been positive in improving support for foster carers.
36. Social workers and managers work effectively alongside the regional adoption agency (RAA), Adoption Connects, to drive forward care planning and support for families. Innovative marketing strategies are used to encourage interest in adoption.
37. Children with a plan for adoption are helped to understand the decisions that are made about them, and supported in a timely, thoughtful and sensitive manner. Children benefit from personalised life-story books and detailed later-life letters. Social workers continue to work collaboratively with birth families

whenever possible to ensure inclusive and child-focused 'wish you well' meetings and that birth families take up the independent support offered by the RAA. High-quality and well-coordinated relational support offered to new adopters helps them to grow and strengthen their bonds with their children.

38. Children in supported accommodation live in homes where their needs can be appropriately met. Social workers have a clear understanding of children's lived experiences, their levels of independence and their expressed preferences. Consequently, children are well matched to supported accommodation, with appropriate wraparound support.
39. A very small number of children live in unregistered children's homes. These homes are used only when there is no alternative. There are appropriate levels of increased oversight by social workers and senior managers of children living in these situations. Central Bedfordshire encourages and supports providers to register with Ofsted.

### **The experiences and progress of care leavers: good**

40. Most care leavers in Central Bedfordshire benefit from strong relationships with their PAs. PAs are tenacious in developing relationships with young people, meaning that most are in regular touch with them, including when they live outside Central Bedfordshire. PAs are caring and develop trusting relationships, which means that young people feel listened to. Care leavers are actively encouraged to continue to accept support up to the age of 25.
41. PAs support care leavers well to maintain their emotional and physical health. PAs are alert to vulnerabilities and are proactive in providing or identifying support and in advocating for young people when needed, for example in accessing adult mental health support. Care leavers are routinely registered with GPs and dentists and are encouraged by their PAs to address their health needs. Conversations are held around substance misuse, with young people guided to supportive resources.
42. Care leavers benefit from a mostly well-developed local offer. Young people are made aware of their entitlements. This information is kept up to date and young people are supported to access the help available. Council tax exemption has not been established as standard, although this is now being addressed.
43. Most care leavers live in suitable accommodation, including staying put with their foster carers when they want to. An independence passport is used to structure preparation for independence. Those in semi-independent accommodation are supported with housing applications and to move into independent accommodation when they are ready. Care leavers benefit from a 'staying close' offer and are well supported into independent living.

44. Transition planning for most children in care is timely. PAs are usually allocated in good time to allow them to develop strong relationships before children leave care.
45. Pathway plans are drawn up collaboratively with young people. They are clear and accessible and translated as required. Plans reflect young people's voices, the work undertaken with them and the progress they are making and set relevant and mostly aspirational actions. They include reference to the local offer and are updated regularly.
46. Most care leavers in custody are visited regularly by their PAs and their needs and vulnerabilities are well supported. There is appropriate preparation for their release from custody.
47. Care leavers who were children seeking sanctuary are supported by PAs who know them well and who build positive relationships. PAs understand young people's cultural needs and the impact of the trauma they have experienced and support them well.
48. When action is needed to keep young people safe, PAs are proactive, for example booking consultations with the child exploitation practitioner to help understand and evaluate risk in order to address it. As a result, most care leavers have their risks and vulnerabilities understood and have plans in place to mitigate them.
49. Many care leavers benefit from advice and support in preparing for employment and in accessing work and training opportunities. The council has developed a limited number of apprenticeships for care leavers but not all PAs are aware of the small number of opportunities that do exist.
50. Most care leavers have the important documents they need for adult life, including a passport, birth certificate and driving licence.
51. Collaborative working between specialist PAs and adult services ensures that the process of identifying, assessing and planning for the needs of care leavers who will need lifelong support begins early and is undertaken jointly with partners and with families. This means that young people are well prepared for their next steps.

## **The impact of leaders on social work practice with children and families: good**

52. Senior leaders work together to ensure that children are the priority across the council. The commitment of the corporate leadership is evident in the political and financial support for children's services in Central Bedfordshire. This is reflected in the protection of frontline children's services and the council making

care leaver a protected characteristic, which provides opportunities to improve outcomes for care leavers.

53. Since the last inspection in 2022, there have been changes in the senior leadership team, including the DCS and lead member. Despite these changes, improvements have continued to be made for children and families in Central Bedfordshire. This has included addressing shortfalls in practice identified at previous inspections about children's advocacy and the quality of direct work with children. Planned developments associated with the children's social care reforms have been introduced, with a restructuring of children's social care. This has resulted in a broader family help offer and greater consistency of worker for children and families. Practitioners were consulted and kept up to date about these changes. The family help model is well understood by practitioners and is being carefully monitored by senior leaders as this develops and embeds to become business as usual.
54. Senior leaders have been responsive to changes in demand. Effective services have been developed to meet the needs of children who are seeking sanctuary. Small specialist teams have been developed to focus on the issues for children at risk of child exploitation and going missing. These new teams support practitioners and partners effectively to understand and address the risks and needs of these vulnerable children.
55. There is variation in the effectiveness of partnership working. Many children and families are being well supported across children's services by partnership working with health and education colleagues, which is making a tangible difference to children's experiences. Partnership work with the Children and Family Court Advisory and Support Service (Cafcass) and the Local Family Justice Board is strong. Where there are challenges, there is engagement at operational and strategic levels to seek to address these, in particular to address the quality and timeliness of contacts from police, probation and the ambulance service.
56. The representation of the views of children in care and care leavers through participation forums is limited. Although there is regular consultation at the corporate parenting board, the CiCC and the care experienced forum (CEF) only involve a small number of children and care leavers. Senior leaders correctly recognise that this is an area requiring development and have plans in place to broaden involvement and introduce events to celebrate children's achievements.
57. A revised and strengthened quality assurance and performance framework is resulting in a wider engagement of practitioners in learning and development. As well as regular meetings and forums, there are targeted workshops and reflective sessions for practitioners. This is supporting improvements in the quality of social work practice.

58. The managers' audit tool encourages a child-focused approach. This helps auditors to identify relevant shortfalls and draw out good practice. Audits are mostly completed collaboratively with practitioners and feedback is regularly sought from children, families and professionals who know the family.
59. Most practitioners describe their induction as supportive and well sequenced. Clarity on expectations, appropriate training, available supervisors and manageable workloads all contribute to new practitioners feeling welcome and confident in their practice.
60. Most practitioners benefit from the broad range of training offered to enable them to remain up to date with their knowledge and practice. Some practitioners said their development needs were well considered but for others, particularly PAs, there is insufficient training to help prepare them for their role.
61. The family practitioner role is new and being carefully developed in the Family Help teams. These workers have limited caseloads and receive regular supervision, and each has a clear development plan to ensure they can support families effectively.
62. Action by leaders has led to significant progress in establishing a stable workforce and manageable workloads. Practitioners feel well supported and describe managers as accessible and approachable. Most receive regular supervision which they find helpful, and which is supporting the progress of children's plans. Service restructuring has successfully reduced the number of changeover points for children and families and there are concrete plans to further improve recruitment and retention. Overall, leaders have created an environment and culture where good practice can thrive.

The Office for Standards in Education, Children's Services and Skills (Ofsted) regulates and inspects to achieve excellence in the care of children and young people, and in education and skills for learners of all ages. It regulates and inspects childcare and children's social care and inspects the Children and Family Court Advisory and Support Service (Cafcass), schools, colleges, initial teacher training, further education and skills, adult and community learning, and education and training in prisons and other secure establishments. It assesses council children's services, and inspects services for children looked after, safeguarding and child protection.

If you would like a copy of this document in a different format, such as large print or Braille, please telephone 0300 123 1231, or email [enquiries@ofsted.gov.uk](mailto:enquiries@ofsted.gov.uk).

You may reuse this information (not including logos) free of charge in any format or medium, under the terms of the Open Government Licence. To view this licence, visit [www.nationalarchives.gov.uk/doc/open-government-licence](http://www.nationalarchives.gov.uk/doc/open-government-licence), write to the Information Policy Team, The National Archives, Kew, London TW9 4DU, or email: [psi@nationalarchives.gsi.gov.uk](mailto:psi@nationalarchives.gsi.gov.uk).

This report is available at <https://reports.ofsted.gov.uk/>.

Interested in our work? You can subscribe to our monthly newsletter for more information and updates: <http://eepurl.com/iTrDn>.

Piccadilly Gate  
Store Street  
Manchester  
M1 2WD

T: 0300 123 1231  
Textphone: 0161 618 8524  
E: [enquiries@ofsted.gov.uk](mailto:enquiries@ofsted.gov.uk)  
W: [www.gov.uk/ofsted](http://www.gov.uk/ofsted)

© Crown copyright 2025